

Robbins et al. v. Travelers Insurance Co.

19 O.R. (2d) 279

ONTARIO

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HIGH COURT OF JUSTICE

KEITH, J.

4TH APRIL 1978.

Insurance -- Accident insurance -- Causation -- Insured dying of heart attack induced by accident -- Insured's heart previously diseased -- Whether accident direct and independent cause of death -- Whether disease contributing cause.

By the terms of an accident insurance policy a certain sum was payable on death resulting from accidental bodily injuries "which are the direct and independent cause of the loss". By an exclusion, any loss caused or contributed to by disease was not to be considered as resulting from injury. The insured died from cardiac arrest induced by a motor vehicle accident. Evidence showed that his heart was previously diseased, but that the arrest was caused by the accident. In an action on the policy, held, the accident was the direct and independent cause of the insured's death, and it could not be said that the disease was the cause, or a contributing cause, of death.

[Jason v. Batten, [1969] 1 Lloyd's Rep. 281, distd; Fidelity & Casualty Co. of New York v. Mitchell (1917), 13 O.W.N. 181, 36 D.L.R. 477, [1917] A.C. 592; Battle v. Fidelity & Casualty Co. of New York (1923), 54 O.L.R. 24 [affd 55 O.L.R. 330]; Caswell v. Powell Duffryn Associated Collieries Ltd., [1940] A.C. 152, refd to]

ACTION on an insurance policy.

Harvey J. Bliss, for plaintiffs.

Ian F. H. Rogers, Q.C., for defendant.

KEITH, J.:-- In this action, the plaintiffs seek recovery of the principal sum, \$50,000 and interest, which they assert the defendant is liable to pay pursuant to the provisions of a personal disability policy issued to the deceased Herbert Henry Robbins in 1966. The insured died on March 26, 1975, as a result of cardiac arrest immediately after being involved in a motor vehicle collision.

An autopsy performed by a highly qualified pathologist revealed that the deceased at the time of death had a severely diseased heart and coronary vascular system and in the words of Dr. John K. Wilson, chief of cardiology at St. Michael's Hospital, Toronto, called as a witness for the defendant, was "a good candidate for cardiac arrest".

It was conceded at the commencement of trial by counsel for the defendant that the policy was in force at the time of the death of Mr. Robbins and that all proof of loss and supporting documents had been duly filed.

The issues to be decided are, firstly, was the loss, that is, the death of the insured, within the policy coverage having regard to all the circumstances, and if so, secondly, is the defendant relieved from liability by reason of a specific exclusion contained in the policy.

The onus of proof rests upon the plaintiffs with respect to the first issue, and if that is satisfied, shifts to the defendant with respect to the second issue.

The relevant terms of the contract of insurance as set out in the policy (omitting inapplicable words) are as follows:

Definitions.

The term "injuries" as used in this Policy means accidental bodily injuries of the Insured which are the direct and independent cause of the loss and occur while this Policy is in force, hereinafter called "such injuries".

Loss of Life -- Accident.

If "such injuries" shall ... result in loss of life of the Insured, the Company will pay the principal sum.

Exclusions.

II For the purposes of this Policy, any loss which is caused or contributed to by (1) disease ... shall not be considered as resulting from injury.

The deceased, who was in his 54th year at the time of his death, was a very successful manufacturer with worldwide interests. Those who knew him well considered him to be in robust good health, with boundless energy. Apart from his business affairs which must have been very demanding, he participated actively in skiing, badminton, swimming and other sports, as he had done all his life. As a young man he was a better than average hockey and baseball player.

In addition to his principal manufacturing plant in Toronto, he had plants in England, West Germany, France and Australia, which he visited frequently. He enjoyed being with friends on frequent social occasions. In short, the impression I have of him from the evidence of his widow, the manager of his plant in West Germany, and a long-time friend and neighbour, is that he enjoyed his work, his travel and his recreational pursuits to the full, despite the fact that his doctor had advised him that an ECG done in February, 1970, showed that he had suffered a "silent" heart attack at some time prior to the examination, and another ECG done in May, 1973, revealed that he had had a second "silent" heart attack in the intervening period. These attacks were evidenced by scarring or fibrosity in separate areas of the heart.

From time to time after unusual exertion he had experienced chest pains which his doctor believed to be angina. No medication had ever been prescribed for this condition, nor, on the evidence, had he been warned that he should modify his lifestyle, except to guard against a tendency to overweight.

His only medication, taken orally, was prescribed to control a borderline diabetic condition which was free of symptoms.

He saw his own doctor for the last time in October, 1974, for a routine check-up. At that time his blood pressure was moderately elevated and pulses were absent in both legs indicating widespread arteriosclerosis.

In the latter part of February or early March, 1975, Mr. Robbins, his wife, and son Paul, then 19 years old, went to England. This was a combined business and pleasure trip involving attendances at the plants in England and West Germany and then proceeding to Switzerland for a skiing holiday. On this holiday, the Robbins family were accompanied by a Mr. Schumacker who was manager of the West German plant, his wife and daughter.

Mr. Schumacker testified that during the entire week in Switzerland, Mr. Robbins exhibited no sign of incapacity or distress, skied, walked a great deal and participated fully in all activities. Mrs. Robbins recalled that her husband had some problems in sleeping, but she attributed this to the unaccustomed altitude. Otherwise she observed nothing unusual.

On March 22nd, the Schumackers returned to Frankfurt and the Robbins family arrived there the following day in time for breakfast with Mr. Schumacker.

On Monday, March 24th, Mr. and Mrs. Robbins and their son were guests at a big party which included dancing.

On Tuesday, March 25th, the Robbins returned to England, having planned Mr. Schumacker's trip to Toronto the following summer. Schumacker said Robbins appeared to be in fine health.

On Wednesday, March 26th, Mr. Robbins spent the morning at his plant, brought business associates back to his hotel for lunch, and continued with a business meeting until about 4:30 that afternoon.

About 5:00 p.m. the family set out to drive to a friend's home in the country for dinner. Robbins had rented a heavy luxury automobile which he was driving. His son Paul was in the left or front passenger seat and Mrs. Robbins was in the rear behind her son. The road to the friend's home was familiar. On the way, Mr. Robbins stopped as required, before crossing a through road. Because of obstructions, it was difficult to see to the left and right before getting partially into the intersection. Mrs. Robbins said her husband thereupon drove slowly forward when without warning, their car was struck broadside by a light car coming from the right. The heavy car in which they were riding was spun clockwise through a full 90degrees before stopping, and the car that hit them came to a stop some 10 to 15 ft. to the rear of their car.

She said she was able to save herself by holding on to the rear of the seat in front of her, but saw her husband thrown about and striking his chest against the steering wheel.

Robbins scrambled over his son, got out the left front door and started towards the other car. He had only gone a few steps when she saw him lying on the ground, contorted as if in a convulsion. The son raced to his father's assistance and attempted mouth-to-mouth resuscitation but to no avail.

On the medical evidence, clinically speaking, Mr. Robbins was dead as a result of cardiac arrest by the time he fell to the ground.

Neither Mrs. Robbins nor her son suffered any injuries, nor as far as is known, did the occupants of the other car although Mrs. Robbins described the impact as heavy, and certainly the right front door of the Robbins car was so severely damaged that it could not be opened.

As the Easter holiday week-end was approaching, the English

authorities acted with dispatch, an autopsy was started on the day following the accident, an inquest promptly held and the body released for burial in Canada.

Doctor Derek Barrowcliffe, the senior consultant pathologist for the South Warwickshire Hospital Group for the past seven years, came from England to testify. His qualifications as an expert witness in the field of pathology are impressive. He has been a qualified pathologist since 1950, and a forensic pathologist for the Home Office in London for 19 years. He received his degree in medicine in 1943, and for some years was the resident for a very senior cardiologist. He was a founder fellow of the Royal College of Pathologists, vice-president of the Forensic Medicine Society and on his own estimate, has performed upwards of 10,000 autopsies.

The post-mortem on Mr. Robbins was started on Thursday, March 27th, the inquest was completed on Saturday, and the body released for burial in Canada. There was no thought of this litigation at that time and at the inquest Dr. Barrowcliffe relied on visual examination without microscope. Fortunately, however, he retained relevant specimens of tissue for detailed examination if required. His original superficial examination revealed no damage to clothing and no bruising on the exterior surface of the body where such commonly occurs, i.e., chest wall, thighs, knees and shins. Nor was there evidence of soft tissue injury in the area of the cervical spine.

A week or two after the inquest he made some microscopic examination of various tissues from the heart, lungs, kidneys, liver and brain, and also the aorta and the prostate and thyroid glands.

When he learned that the defendant was resisting the claim of the plaintiffs and was asked for further information, he was able to make a much more detailed microscopic study of the tissues he had retained.

He found first of all, a marrow embolus in one slice of lung tissue which indicated injury to a bone, probably a hairline fracture of a rib, which had permitted a particle of bone

marrow to be forced into the blood stream, passing through the heart, entering the lung and becoming wedged in a small blood vessel in the lung. In his opinion, which I accept, a bone marrow embolus must originate from trauma.

Next he found contusions or haemorrhages of the lungs, most marked in the right upper lobe and left lower lobe and up to two centimetres in diameter. In his opinion, these were probably caused by the same trauma responsible for the rib fracture that caused the marrow embolus.

The lung contusions, in his opinion, caused firstly a degree of shock, followed, as in this case, where there was extensive disruption of blood vessels, by the cutting off of a significant percentage of the oxygen supply to the body, and most particularly the heart muscle itself.

He then observed evidence of a haemorrhage surrounding the arch of the aorta which, of course, is the principal blood vessel leaving the heart. He described how the heart is suspended in the chest wall from the arch of the aorta, and stated that in his opinion, the haemorrhage he observed was caused by the heart being shaken inside the chest cavity.

In the result, it was Dr. Barrowcliffe's unshaken opinion that Mr. Robbins' heart stopped as a direct result of the impact of the collision which, having regard to the marrow embolus, the damage to the blood vessels in the lungs and the arch of the aorta, must have been severe.

He also discussed the effects of such a blow on the nervous system, the release of excessive amounts of adrenalins and noradrenalins, interference with the rhythm of the heart and other related matters.

In conclusion he made the following statements as to the cause of death: Mr. Robbins died as a result of an acute cardiac arrest which was caused by the trauma suffered in the accident and the accident itself. These induced a combination of several mechanisms acting alone and in concert, such as: contusion of the lungs, tugging and twisting of the heart in

the region of the aorta, compression of the heart between steering wheel and spine, sensitivity to lack of oxygen, abnormal rhythm, haemorrhage around the aorta, excess adrenals released emotionally by the accident, and inhalation of stomach content which interfered with the function of the lungs generally.

In his opinion, in the absence of trauma, Mr. Robbins' heart might well have continued to function for many years, and his pre-existing condition was not a factor in his death. While he agreed that a healthy heart is better equipped to resist trauma, it is still susceptible to cardiac arrest in similar circumstances.

Doctor Wilson, to whom I have referred, was called as a witness for the defence. He was in no position to challenge the opinion of the pathologist and, in effect, did not do so.

It is on this evidence that the precise language of the policy must be considered with respect to coverage in the first place, and exclusion in the second place.

As to the first issue, the critical words are contained in the definition of "injuries" and they are the words "direct and independent cause". Counsel for the defendant frankly admitted that the high authority he had to meet was that of the Privy Council in the case of *Fidelity & Casualty Co. of New York v. Mitchell* (1917), 13 O.W.N. 181, 36 D.L.R. 477, [1917] A.C. 592. In that case the appellant insurance company insured the respondent against "Bodily injury sustained during the term of one year from ... the day that this policy is dated, through accidental means . . . and resulting directly, independently and exclusively of all other causes ...", upon proof of which the insured was to be paid a weekly indemnity in case of total disability resulting from such bodily injury. The insured had suffered from a tubercular infection some years before the accident which gave rise to this litigation. The tubercular lesion in the lung had completely healed and the tuberculosis in his system was latent. He severely sprained his wrist in an accident and the latent tuberculosis became active and prevented his wrist from healing. Lord Dunedin, in delivering

the opinion of the Privy Council, affirming the trial judgment and the Court of Appeal for Ontario in favour of the insured, said at p. 182 O.W.N., p. 481 D.L.R., p. 597 A.C.:

Prior to the accident there was only a potestative tuberculous tendency: after it, and owing to it, there was a tuberculous condition. In other words, the accident had a double effect: it sprained the tendons, and it induced the tuberculous condition. These two things acted together, and were the reason of the continuing disability; but while they are both ingredients of the disabled condition, there had been and is, on the true construction of the policy, only one cause, viz., the accident.

The insuring agreement in that case contained the word "exclusively" which does not appear in the present case. In my opinion, the insuring agreement is much less favourable to the insurer than the agreement in the Mitchell case. It is to be noted that the policy in the Mitchell case contained no exclusion, and the case fell to be determined solely on the language of the insuring agreement.

Counsel for the defendant relies on the case of Jason v. Batten, [1969] 1 Lloyd's Rep. 281, a decision of a single Judge. The insuring agreement in the accident policy in question read as follows: "Sustain in any accident bodily injury resulting in and being -- independently of all other causes -- the exclusive direct and immediate cause of the ... disablement of the Insured Person". The plaintiff, who had a serious existing arterial condition, suffered a coronary thrombosis shortly after a motor accident. The learned trial Judge assumed for the purposes of his judgment, that the circumstances brought the plaintiff within the terms of the insuring agreement. The case is, therefore, of no assistance to the defendant on the first issue.

The plaintiff Jason failed, however, because of a very strongly worded exclusionary clause which read as follows: "No benefit shall be payable under this policy in respect of Death, Injury or Disablement directly or indirectly caused by or arising or resulting from or traceable to ... any physical

defect or infirmity which existed prior to the accident" (emphasis added).

I will refer back to this clause when dealing with the second issue.

I refer next to the case of *Battle v. Fidelity & Casualty Co.* of New York (1923), 54 O.L.R. 24 [affirmed 55 O.L.R. 330]. The policy in that case was identical with the policy in the Mitchell case, that is, the insuring agreement was in the same language and there was no exclusionary clause. The plaintiff suffered from several serious pre-existing physical conditions. He was involved in an accident as a result of which these conditions became active, an arm became gangrenous and had to be amputated. Rose, J. (as he then was), followed the Mitchell case and in doing so said, at pp. 30-1:

The second essential, under the terms of the policy, is that the disability or death mentioned in the insuring clause, or the amputation, etc., mentioned in the schedule of injuries, shall result directly and "independently and exclusively of all other causes" from the bodily injury. The defendants contended that this essential is not met, in that the arterio-sclerosis or the diabetes or both of them caused, or was or were a cause or causes of, the gangrene. That gangrene would not have come from either the injury to the foot or the injury to the arm if there had been no disease is, as I have stated, quite clear; and that it did not come from the disease alone is, in my opinion (as I have also stated), the fair result of the whole of the evidence. Was, then, the disease a "cause" within the meaning of that word as used in the policy, or was it merely a "condition," without which the active cause--the injury--would have been ineffective to produce the result which gives rise to the claim? Similar questions have been discussed in many cases arising upon policies worded somewhat like the policy sued upon here. Many of those cases are discussed in a book to which Mr. Grant referred me, "Accidental Means," by Mr. Martin P. Cornelius (1916), but I do not think it necessary to go through them, for I think that the fair result of the Mitchell case, above referred to, 35 O.L.R. 280, 37 O.L.R.

335, [1917] A.C. 592, a decision upon a policy in exactly the same form as the present one, is that the disease was a condition sine qua non, rather than a cause. It is true that in Mitchell's case "the tuberculosis was latent, and would have remained harmless had it not been for the accident" (35 O.L.R. at p. 286)--that "prior to the accident there was only a potestative tuberculous tendency" ([1917] A.C. at p. 597)--whereas, in the present case the arterio-sclerosis was a progressive disease which might have caused gangrene, at some time. But gangrene had not come from the disease--it has not occurred in the right leg or the right arm, which, as regards circulation of blood, were presumably in the same state as were the leg and arm which had to be amputated--and it would not have occurred when it did in the left leg or arm but for the injuries. Therefore, while there is a distinction between the two cases, and while much that was said in Mitchell's case is inapplicable to the present case, my opinion is that the principle of the Mitchell case is applicable here, and that it ought to be held that the bodily injury sustained by the plaintiff did result, "directly, independently and exclusively of all other causes," in the disability and the amputations.

Based on Dr. Barrowcliffe's evidence, I find that the late Mr. Robbins died of injuries as defined in the policy in accordance with the interpretation put on more restrictive words in the Mitchell case and the Battle case.

The question then, is whether or not the plaintiff is barred from recovering because of the language of the exclusion in the policy. It will, no doubt, be convenient to repeat that language:

Exclusions.

II For the purposes of this Policy, any loss which is caused or contributed to by (1) disease ... shall not be considered as resulting from injury.

(Emphasis added.)

It is clear from Dr. Barrowcliffe's evidence that Mr. Robbins' death was not caused by disease. To succeed, the defendant must rely on the proposition that the pre-existing cardio-vascular condition which created a diseased heart "contributed" to the death by giving to the word "contributed" a meaning not associated with causation.

The phrase "caused or contributed to" is used in the Negligence Act, R.S.O. 1970, c. 296, s. 2. There can be no doubt that as used in the statute they are words of art with a well understood legal meaning.

In *Caswell v. Powell Duffryn Associated Collieries Ltd.*, [1940] A.C. 152, Lord Atkin said, at p. 165: "I find it impossible to divorce any theory of contributory negligence from the concept of causation."

In my opinion the words of the exclusion in question may be properly amplified in this way: "For the purposes of this Policy any loss which is caused by disease or to which disease is a contributing cause ... shall not be considered as resulting from injury."

The evidence of Dr. Barrowcliffe which I have accepted, was unqualified to the effect that true, Mr. Robbins had a diseased heart which was an underlying condition and which might well have led to his death at some time, but the cause of death was injuries suffered in the accident which, together with emotional stress accompanied by the release of excessive quantities of adrenalins and noradrenalins and an arhythmic heartbeat leading to fibrillation resulted in acute cardiac arrest.

The exclusionary clause in *Jason v. Batten*, *supra*, was very much wider in its scope than in the policy issued by the defendant in the present case. It speaks not only of direct causation but also "indirect" causation, and in addition uses the phrase "traceable to" with respect to a pre-existing physical defect or infirmity.

Finally, if there is any doubt or ambiguity in the meaning of the words "'contributed to" in the policy in this case (which I do not think exists) such doubt or ambiguity is properly resolved contra proferentem.

I do not find it necessary to refer to the many other cases cited by counsel in which exclusionary clauses were considered, since in all such cases the words "directly or indirectly" or other broadening expressions, were employed by the draftsman of the clause and affected the judgments of the Courts.

In the result, therefore, the plaintiffs will have judgment for the sum of \$50,000 together with interest thereon at the prime rate from time to time from March 26, 1975, as determined and published by the Bank of Canada.

The plaintiffs are also entitled to their costs of this action including all costs and expenses of and incidental to the attendance of Dr. Barrowcliffe as an essential witness.

Judgment for plaintiffs.